

ਕੇਂਦਰੀ ਗੁਰਦੁਆਰਾ ਸ੍ਰੀ ਗੁਰੂ ਸਿੰਘ ਸਭਾ
ਚੈਰੀਟੇਬਲ ਲੈਬੋਰੇਟਰੀ
ਰਾਜਪੁਰਾ ਟਾਊਨ, ਜਿਲ੍ਹਾ ਪਟਿਆਲਾ (ਪੰਜਾਬ)



Name: Mrs. PRABHJOT KAUR UHID.: 105192
Age/Gender: 28 Y/Female Registered: 18/Aug/2021 07:14AM
Lab No: 022108180002 Visit Type:
BarcodeNo: 10137001 Referred By.: Self
Register No: 2/18 Panel: RAJPURA
Mobile: 6239939398 Address:
Test Name: THYROID PROFILE,Haemoglobin (Hb)

Net Amt: Rs.150 Paid Amt: Rs.150 Balance Amt: Rs.0
Report Status: Final

HAEMATOLOGY

Test Name	Value	Unit	Bio Ref.Interval
Haemoglobin (Hb)			
Haemoglobin (HB)	12.1	g/dL	11.5 - 15.0
Method -Colorimetric method			
PCV / Haematocrit	38.9	%	35.0 - 45.0
Method -Calculated			
Red Blood Cell (RBC)	4.1	Million/mm3	3.8 - 4.8
Method -Electrical impedance			
Mean Corp. Volume (MCV)	94.2	fl	80.0 - 100.0
Method -Electrical impedance			
Mean Corp. HB (MCH)	29.3	pg	27-32
Method -Calculated			
Mean Corp. HB Con. (MCHC)	31.1 ^L	gm/dL	31.5-34.5
Method -Calculated			
Red Cell Distribution Width-CV	18.7 ^H	%	11.6-14.0
Method -Electrical impedance			
Red Cell Distribution Width-SD	62.6 ^H		35.0 - 56.0
Method -Electrical impedance			
Test performed on BC_DxH_800			



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8567- 8567- 35

Name: Mrs. PRABHJOT KAUR UHID.: 105192
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IMMUNOLOGY

Test Name Value Unit Bio Ref.Interval

THYROID PROFILE

T3 (Total Triiodothyronine) ,Serum	1.13	ng/mL	0.60 - 1.81
Method -ECLIA			
T4 (Total Thyroxine), Serum	7.79	ug/dL	5.10 - 14.10
Method -ECLIA			
TSH (Thyroid Stimulating Hormone), Serum (3rd generation ultrasensitive)	2.26	uIU/ml	0.20 - 4.70
Method -ECLIA			

In case of Increased TSH (4.2 to 10.0).

Advised a repeat **TSH after 1 month**, If the patient is not on Thyroid Medication, to rule out transient hypothyroidism. Which may be due to subclinical viral infection, Sleep deprivation, medication etc.

Note:

- 1.Total T3 & T4 levels measure the hormone which is in the bound form and is not available to most tissues.
2. Severe systemic illness affects the thyroid binding proteins and can falsely alter Total T4 levels in the absence of a primary thyroid disease. Hence Free T3 & T4 levels are recommended for accurate assessment of thyroid dysfunction.

Clinical Use:

Diagnose Hypothyroidism and Hyperthyroidism when overt and / or due to pituitary or hypothalamic disease.

Increased Levels: Hyperthyroidism, Increased TBG, Familial dysalbuminemic hyperthyroxinemia, Increased Transthyretin, Estrogen therapy, Pregnancy.

Decreased Levels: Primary hypothyroidism, Pituitary TSH deficiency, Hypothalamic TRH deficiency, Non thyroidal illness, Decreased TBG.

T4 (Total Thyroxine)

Adults :		
Paediatric :	1 - 3 days :	1.2 - 19.9
Paediatric :	1 week :	6.0 - 15.9
Paediatric :	1 - 12 months :	6.1 - 14.9
Paediatric :	1 - 3 yr :	6.8 - 13.5
Paediatric :	3 - 10 yr :	5.5 - 12.8

Note: TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm The



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Helpline Number
8567- 8567- 35

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variation is of the order of 50% .hence time of the day has influence on the measured serum TSH concentrations.

Clinical Use:

Diagnose Hypothyroidism and Hyperthyroidism
Monitor T4 replacement or T4 suppressive therapy
Quantify TSH levels in the subnormal range

Increased Levels: Primary hypothyroidism, Subclinical hypothyroidism, TSH dependent, Hyperthyroidism

Thyroid hormone resistance

Decreased Levels: Graves disease, Autonomous thyroid hormone secretion, TSH deficiency

TSH Reference Values:-			
Paediatric:		Pregnancy:	
Cord Blood	: 2.30 13.2	1st Trimester	: 0.10 2.50
1 - 04days	: 1.00 39.00	2nd Trimester	: 0.20 3.00
2 - 20 weeks	: 1.70 9.10	3rd Trimester	: 0.30 3.00

Test performed on Cobas_e601

*** End Of Report ***

Note: The reports are strictly for the use of Medical Practitioners and are not medical diagnosis as such.

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